

Bluish Skin or Body Part (Cyanosis) - Male (Child)

Algorithm 1097 | Child (0-17 years) | Gender at birth: Male

Definition

- UAT assessment structure for a reported blue, grey, unusually pale, or otherwise abnormal colour. It separates central/generalized colour change (lips, tongue, oral mucosa, face, or widespread skin), which may reflect respiratory, cardiac, circulatory, sepsis, or hypothermia emergencies, from a colour change confined to a finger, toe, hand, or foot, which may reflect cold-related peripheral vasoconstriction, vascular compromise, compression, or trauma. Colour reports are not a diagnosis and must be interpreted with breathing, behaviour, circulation, temperature, injury, and the person's usual appearance.

- Child patient.
- Male.

Initial assessment

1. [OPEN_TEXT] What is the person's exact age, and is this colour new or different from their documented usual appearance or congenital heart-disease plan?
2. [LOCATION] Exactly where is the colour change: lips, tongue or inside the mouth, face, widespread skin, or only one or more fingers, toes, hands, or feet? Is it one-sided or on both sides?
3. [YES_NO] Is the person fully responsive and breathing normally? For a baby or child, are they difficult to wake, floppy, confused, unable to feed/cry/speak normally, grunting, flaring the nostrils, drawing in under/between the ribs, having pauses, or breathing unusually fast, slow, or irregularly?
4. [YES_NO] Is there severe breathlessness, choking, chest pain, collapse/fainting, seizure, confusion, cold or clammy skin, fever or suspected serious infection, or rapidly worsening illness?
5. [YES_NO] If confined to a limb or digit, did it start suddenly, and is there severe pain, coldness, numbness, weakness, swelling, injury, a tight cast/bandage/jewellery, or inability to move the limb?
6. [YES_NO] Was there significant cold or wet exposure, a measured temperature below 35 C, or in a baby: cold trunk, unusual quietness, sleepiness, floppiness, or refusal to feed?
7. [CAREGIVER_OBSERVATION] What change has a safe parent or caregiver observed in behaviour, alertness, eating or drinking, sleep, school or play, and normal activity?

Triage and redirect questions

1. Disposition level 100

Is there a new blue, grey, very pale, or markedly abnormal colour of the lips, tongue/inside the mouth, face, or widespread skin; uncertainty about whether the change is central or only peripheral; or any colour change with abnormal breathing, choking, chest pain, collapse, seizure, confusion, reduced responsiveness, cold/clammy appearance, suspected sepsis or hypothermia? In a baby or child, include floppiness, difficulty waking, poor feeding, weak/abnormal cry, grunting, nasal flaring, chest indrawing, breathing pauses, or unusually fast, slow, or irregular breathing.

Central or generalized abnormal colour can indicate inadequate oxygenation or circulation from respiratory, cardiac, infectious, shock, or temperature-related illness. Resuscitation Council UK treats abnormal body colour together with abnormal behaviour or breathing as a sign of possible critical illness in children. Colour can be difficult to assess remotely and across skin tones: compare with the person's usual colour and inspect lips, tongue/oral mucosa, nail beds, palms, and soles, but never use apparent colour alone to dismiss abnormal breathing or behaviour.

2. Disposition level 100

Is the colour change confined to a finger, toe, hand, foot, arm, or leg but sudden or rapidly worsening, with severe pain, marked coldness, numbness, weakness, inability to move, major swelling, significant trauma, or a tight cast/bandage or other compression?

A sudden painful, cold, numb, weak, injured, compressed, or rapidly changing limb may have threatened blood flow or serious injury and requires emergency in-person assessment; it must not be downgraded because the lips or tongue look normal.

3. Disposition level 70

Is the colour change definitely confined to fingers, toes, hands, or feet, with normal lips/tongue/inside the mouth, normal responsiveness and breathing, no emergency limb features, but it is persistent, recurrent, unexplained, or not returning toward the person's usual colour after shelter and gentle whole-body warming?

A clearly isolated peripheral colour change without systemic or limb-threat features is not automatically central cyanosis, but persistent, recurrent, or unexplained change still needs prompt in-person assessment. The approved Qatar non-emergency route and timing remain a local governance decision.

Advice

Qatar emergency response for central or systemic colour change

- Call Qatar emergency services on 999 now and follow the call-taker's instructions. Keep the person with you and allow the position in which breathing is easiest; do not make them walk or drive. Use prescribed oxygen or rescue treatment only according to their personal plan. If they become unresponsive and are not breathing normally, start age-appropriate CPR as directed by 999 and use an AED if available. Do not give food, drink, or unprescribed medicine. If cold or wet, move to shelter, remove wet clothing if practical, cover with dry layers, and handle gently; do not rub the limbs or use a hot bath, heat lamp, or hot-water bottle. Do not delay 999 while trying to confirm colour by video or pulse oximetry. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.
- Call Back If: abnormal lips, tongue, oral mucosa, face, or widespread colour; difficulty breathing or choking; chest pain or collapse; confusion, seizure, floppiness, or reduced responsiveness; infant poor feeding or abnormal breathing; suspected sepsis, shock, or hypothermia.

Emergency response for threatened limb or serious injury

- Call Qatar emergency services on 999 for a suddenly painful, cold, numb, weak, immobile, severely swollen, injured, compressed, or rapidly discolouring limb or digit. Keep the person and affected part at rest. If safe, remove an obvious external constricting item such as a ring before swelling worsens, but do not cut off or alter a cast or medical dressing unless instructed by emergency services. Do not rub, massage, directly heat, tightly wrap, or compress the part, and do not give food, drink, or unprescribed medicine. Do not drive. This is a direct emergency pathway; do not redirect to an unavailable limb-pain, vascular, trauma, or cold-injury protocol. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.
- Call Back If: rapidly worsening colour; severe pain; cold, numb, weak, or immobile limb; major swelling or injury; tight cast or compression; new systemic or breathing symptom.

Prompt in-person review for isolated peripheral colour change

- Arrange prompt same-day in-person assessment through the Qatar route approved by local clinical governance. While arranging care, move from cold or wet conditions, remove wet clothing, and warm the whole person gradually with dry layers; do not rub or massage the affected part or apply direct heat. Remove only an obvious external constricting item if safe. Remote appearance cannot confirm the cause. Call 999 immediately if colour involves the lips, tongue, inside the mouth, face, or becomes widespread; if breathing, responsiveness, feeding, chest symptoms, temperature, or general condition is abnormal; or if the limb becomes painful, cold, numb, weak, swollen, injured, or rapidly worse. Do not use a compound redirect.
- Call Back If: central or widespread colour change; abnormal breathing or responsiveness; chest pain, collapse, fever, or serious illness; infant feeding or behaviour change; painful, cold, numb, weak, swollen, or injured limb.

First aid

- Call Qatar emergency services on 999 now and follow the call-taker's instructions. Keep the person with you and allow the position in which breathing is easiest; do not make them walk or drive. Use prescribed oxygen or rescue treatment only according to their personal plan. If they become unresponsive and are not breathing normally, start age-appropriate CPR as directed by 999 and use an AED if available. Do not give food, drink, or unprescribed medicine. If cold or wet, move to shelter, remove wet clothing if practical, cover with dry layers, and handle gently; do not rub the limbs or use a hot bath, heat lamp, or hot-water bottle. Do not delay 999 while trying to confirm colour by video or pulse oximetry. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.
- Call Qatar emergency services on 999 for a suddenly painful, cold, numb, weak, immobile, severely swollen, injured, compressed, or rapidly discolouring limb or digit. Keep the person and affected part at rest. If safe, remove an obvious external constricting item such as a ring before swelling worsens, but do not cut off or alter a cast or medical dressing unless instructed by emergency services. Do not rub, massage, directly heat, tightly wrap, or compress the part, and do not give food, drink, or unprescribed medicine. Do not drive. This is a direct emergency pathway; do not redirect to an unavailable limb-pain, vascular, trauma, or cold-injury protocol. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.

References

- Hamad Medical Corporation, "Life-Threatening Medical Emergency", <https://hamad.qa/EN/Emergency/Pages/Life-ThreateningMedicalEmergency.aspx> (accessed 2026-07-25)
- Resuscitation Council UK, "Paediatric Life Support (basic and advanced): 2025 Guidelines", <https://www.resus.org.uk/professional-library/2025-resuscitation-guidelines/paediatric-basic-life-support-guidelines> (published 27 October 2025)
- NHS.UK, "Shortness of breath", <https://www.nhs.uk/conditions/shortness-of-breath/> (page last reviewed 30 January 2024)
- NHS.UK, "Bronchiolitis", <https://www.nhs.uk/conditions/bronchiolitis/> (accessed 2026-07-25)
- NHS.UK, "Hypothermia", <https://www.nhs.uk/conditions/hypothermia/> (page last reviewed 09 June 2023)

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